

Asthma– Summary

Definition of good asthma control (assess at each follow up visit):

- Daytime symptoms: ≤ 2 days/week, nighttime symptoms: < 1 night/week and mild, normal physical activity, mild and infrequent exacerbations, no absence from school/work, ≤ 2 doses of reliever/week

Non-Pharmacological Treatment:

- Avoid precipitating factors (i.e. occupational irritants, environmental allergens, etc.), smoking cessation, treat modifiable comorbidities (i.e. obesity, anxiety, depression, GERD), hyposensitization therapy to allergens, provide asthma education and develop a written action plan (for all patients)
- Provide the following plans to all patients:
 - Education plan:** asthma pathophysiology, triggers, “good control” parameters, exacerbation risk factors, reliever vs. Control, medication safety, inhaler technique
 - Written action plan:** how/when to assess control, controller adherence, environmental changes, management of worsening symptoms, when to seek help

Pharmacological Treatment:

(reassess every 6-12 weeks to step up therapy and every 3 months to step down)

- If the patient is a child aged 6-11 years-old AND:**
 - STEP 1: Symptoms occur $< 2x$ /month:** low-dose ICS taken whenever SABA PRN is taken
 - Alternative: daily low dose ICS with SABA PRN
 - STEP 2: Symptoms $> 2x$ /month but less than daily:** low-dose ICS/formoterol PRN (preferred) OR daily low dose ICS with SABA PRN
 - Alternative: LTRA or low-dose ICS taken whenever SABA PRN is taken
 - STEP 3: Symptoms most days of the week, any nocturnal awakening weekly:** medium-dose ICS daily with SABA PRN OR low-dose ICS/LABA daily with SABA PRN
 - Alternative: low-dose ICS and LTRA + SABA PRN
 - STEP 4/5: Symptoms most days of the week, any nocturnal awakening, low lung function:** medium-dose ICS/LABA daily with SABA PRN
 - Alternative: add LTRA, consider LAMA, refer for expert advice
- If the patient is ≥ 12 years-old AND:**
 - STEP 1/2: Symptoms $< 4x$ /week, no nocturnal symptoms:** low-dose ICS/formoterol PRN (preferred)
 - Alternative: Low-dose ICS daily + SABA prn; low-dose ICS taken whenever SABA PRN is taken (only if symptoms $< 2x$ /month); LTRA
 - STEP 3: Symptoms most days, any nocturnal wakening weekly:** low-dose ICS/LABA daily (incl bud/form) + SABA PRN OR bud/form
 - Alternatives: low-dose ICS/LABA OR low-dose ICS daily with LTRA OR increase to medium-dose ICS
 - STEP 4: Daily symptoms, any nocturnal wakening weekly, decreased lung function:** medium-dose ICS/LABA daily (incl bud/form) + SABA PRN OR bud/form
 - Alternatives: add LAMA OR LTRA OR increase to high-dose ICS
 - STEP 5: Uncontrolled despite above management:** add LAMA to medium dose ICS/LABA (incl bud/form) + SABA PRN OR bud/form

- Options for add-on: high-dose ICS-LABA OR anti-IgE/IL5/5R/IL4R/TSLP OR short course oral corticosteroid + REFER to specialist

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